

Rapid Response and Code Blue (Cardiopulmonary Resuscitation)

Policy Number	POL-CLN-040	Owner / Department	Resuscitation Committee / Nursing
Effective Date	2024-02-01	Version	3.1
Last Revised	2026-03-12	Next Review	2028-03-12
Approved By	Code Blue / Resuscitation Committee	Applies To	All clinical staff and first responders in all inpatient and outpatient areas of the facility

1. Purpose

To establish a standardized, time-critical response to cardiopulmonary arrest (Code Blue) and to clinical deterioration that has not yet progressed to arrest (Rapid Response), so that high-quality basic and advanced life support reaches the patient without delay. This policy operationalizes the current American Heart Association (AHA) BLS and ACLS guidelines.

2. Scope

Applies to any patient, visitor, or staff member found pulseless and/or apneic (Code Blue) or showing signs of acute deterioration short of arrest (Rapid Response Team, RRT). Any staff member may activate either response; activation is never to be delayed for supervisory approval.

3. Activation Criteria

Call a **Code Blue** (dial 55) for unresponsiveness with absent or abnormal breathing and no definite pulse within 10 seconds. Call the **Rapid Response Team** (dial 66) for any of the following, or for any staff member who is simply “worried” about the patient:

- Acute change in heart rate < 40 or > 130 bpm.
- Systolic blood pressure < 90 mmHg.
- Respiratory rate < 8 or > 28 breaths/min.
- SpO2 < 90% despite supplemental oxygen.
- Acute change in level of consciousness or new seizure.
- A NEWS2 aggregate score ≥ 7, or any single parameter scoring 3 (see POL-CLN-041).

IMPORTANT — If the patient is pulseless and/or not breathing normally, do not wait for the team: call Code Blue (dial 55), start chest compressions immediately at 100–120/min and a depth of 2–2.4 inches (5–6 cm), and attach the AED / defibrillator as soon as it arrives. Every minute without CPR and defibrillation reduces survival by roughly 7–10%.

4. Procedure — First Responder

1. Confirm unresponsiveness and shout for help; activate Code Blue (dial 55) and note the time.

2. Begin high-quality CPR: compressions 100–120/min, depth 5–6 cm, full chest recoil, minimize interruptions (< 10 seconds).
3. Open the airway and deliver ventilations at a 30:2 compression-to-ventilation ratio until an advanced airway is placed; thereafter ventilate once every 6 seconds.
4. Attach pads and analyze rhythm as soon as the defibrillator arrives; defibrillate a shockable rhythm (VF/pulseless VT) without delay, then resume compressions immediately.
5. Rotate the compressor every 2 minutes (every rhythm check) to limit fatigue.
6. Follow the ACLS cardiac-arrest algorithm: epinephrine 1 mg IV/IO every 3–5 minutes; amiodarone 300 mg (then 150 mg) for refractory VF/pVT; and treat reversible causes (the Hs and Ts).

5. Crash Cart and Equipment

- Crash carts are checked and lock-sealed every shift; a broken seal mandates a full inventory and reseal.
- Defibrillator self-test and battery are verified each shift and logged.
- A backboard, bag-valve-mask, and oxygen source are immediately available at every care location.

6. Post-Event Documentation and Debrief

- Complete the code record / resuscitation flowsheet in real time, including times of arrest, CPR start, first shock, drugs, and return of spontaneous circulation (ROSC) or termination.
- Conduct a hot debrief with the team immediately after the event.
- Restock and reseal the crash cart before it returns to service.
- Submit an event report in the safety system per POL-RM-013.

7. Severity & Reporting

Classify and report every resuscitation event per the facility-wide severity scale in POL-RM-013 (SEV-1 catastrophic/sentinel; SEV-2 major; SEV-3 minor/no harm; Near Miss).

Severity	Example	Notify / timeframe
SEV-1	Unrecognized deterioration or failure to rescue leading to death or permanent harm (sentinel event); delayed CPR or missing/failed defibrillator.	Attending, Administrator-on-Call, and Risk Management within 1 hour.
SEV-2	Resuscitation with delayed response or equipment gap but patient recovered with major temporary harm.	Unit manager and Risk Management within 4 hours.
SEV-3	Minor process deviation (e.g. late cart restock) with no patient harm.	Unit manager within 24 hours via event report.
Near Miss	Crash cart seal/equipment failure caught before a code; deterioration intercepted in time.	Event report within 24 hours.

8. References

- American Heart Association, Guidelines for CPR and Emergency Cardiovascular Care (BLS and ACLS).
- AHA Chain of Survival and cardiac-arrest algorithms.
- Related: POL-CLN-041, POL-RM-013.

